

QC Pathologist:

TCGA-AD-6548

FINAL PATHOLOGIC DIAGNOSIS:

Right colectomy with attached portion of terminal ileum:

Two carcinomas of colon.

Tumor Characteristics:

1. Histologic type for both: Adenocarcinoma.
2. Histologic grade for both: Moderately differentiated.
3. Tumor site for both: Splenic flexure area.
4. Tumor size: Largest: 2.9 x 2.6 x 0.5 cm, smallest 1.6 x 1.4 x 0.3 cm.
5. Lymphovascular space invasion: Not identified
6. Microscopic tumor extension:
  - a. Largest tumor exhibits invasion not quite completely through the muscularis propria.
  - b. Smallest tumor does not invade muscularis propria.
7. Treatment effect: None identified.

Surgical Margin Status:

1. Proximal margin nearest to largest tumor: 14.3 cm.
2. Distal margin nearest to smaller tumor: Approximately 4.0 cm.
3. Circumferential (radial) margin which is closest to largest tumor: 0.4 cm.
4. Distance of tumor from closest margin: 0.4 cm.

Lymph Node Status:

1. Total number of lymph nodes received: 12.
2. Total number of lymph nodes containing metastatic carcinoma: None (0/12).

Other:

1. Other significant findings: A separate tubular adenoma without high grade dysplasia is noted.
2. pTNM stage: pT2, N0 (largest tumor). pT1, N0 (smallest tumor).

COMMENTS:

CLINICAL HISTORY:

Preoperative Diagnosis:

Postoperative Diagnosis:

Symptoms/Radiologic Findings:

SPECIMENS:

Right colon

CGA-AD-6548

GROSS DESCRIPTION:

Received in formalin and labeled [REDACTED] and #1 right colon is an 18.1 cm segment of previously opened colon with an attached 2.6 cm segment of small bowel, a moderate amount of pericolic fat, and a 16.6 x 5.3 x 0.7 cm portion of omentum. The appendix is not present.

The serosa is smooth and tan-pink. There is a 2.9 x 2.6 cm granular tan-pink mucosal mass with raised borders, 5.8 cm, 7.9 cm, and 14.3 cm from the distal margin; ileocecal valve, and proximal margins, respectively. The cut surface of the mass is gray-white and appears to extend into the muscularis to a depth of 0.2 cm. The mass does not grossly appear to involve the pericolic fat.

At 1.3 cm distal to the mass, is a 1.6 x 1.4 cm, tan mucosal polyp, 4.1 cm from the distal margin. The polyp does not appear to extend into the underlying muscularis.

The remainder of the mucosa is tan with the normal folds, with a 1.3 x 0.6 cm, ill-defined, firm, tan nodule in the cecal pouch. No additional obvious mucosal lesions are identified.

The proximal and distal inner circumferences average 4.5 cm and 3.8 cm, respectively, and the wall thickness averages 0.3 cm. There are multiple irregular, tan-yellow firm tissues consistent with probable lymph node, ranging from 0.2 x 0.2 x 0.1 cm to 0.7 x 0.5 x 0.4 cm.

The specimen is inked, sectioned, and representative sections are submitted as labeled: 1, proximal and distal margins; 2, ileocecal valve; 3 and 4, mass to inked serosa (perpendicular); 5, mass to uninvolved mucosa; 6 and 7, mass; 8-11, mucosal polyp, entirely submitted; 12, nodule in cecum, entirely submitted; 13, omentum; 14-16, five whole possible lymph nodes in each; 17 and 18, three whole probable lymph nodes in each. The blocks are labeled

Also received in the same container are green, yellow and blue cassettes labeled [REDACTED] for genomic research study.

1 or more Final Results Received